

Software Requirements Specification

Open Clinical Record

Open Clinical Record Internship Project

Requirements Baseline Draft — September 2026

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1 Document Status

This document defines the first complete Software Requirements Specification (SRS) draft for the Open Clinical Record internship project. It consolidates the current project scope, clinical workflow discovery, research findings, preliminary clinical validation, functional requirements, non-functional requirements, business rules, constraints, and verification expectations.

The document is intended to provide a traceable foundation for architecture and data-model decisions. Requirements may still be refined after mentor and stakeholder review; such changes should be recorded rather than silently changing the baseline.

Item	Value
Version	1.0 — first complete SRS draft
Status	Draft for review and baseline
Product	Open Clinical Record
Product type	Extensible Electronic Medical Record (EMR) platform
Primary scope	Patient chart, medical records/reports, appointments and basic encounter workflow
Validation evidence	One preliminary Doctor/Clinician survey response; further validation remains desirable
Next lifecycle stage	Architecture and data-model design after requirements review

2 Introduction

Open Clinical Record is an extensible Electronic Medical Record (EMR) platform intended to provide a clear and maintainable foundation for common clinical workflows. The internship MVP deliberately focuses on three connected areas: the patient chart, medical records and reports, and appointment/visit management.

The system is not intended to be a complete hospital information system in the first release. The requirements therefore prioritize a coherent clinical workflow, reliable record handling, role-based access, and a design that can be expanded later without unnecessarily coupling future modules to the MVP.

2.1 Purpose

The purpose of this SRS is to define what the MVP shall do, the quality constraints it shall satisfy, and the rules that govern important clinical behavior. The SRS also provides a traceability basis between discovery findings, requirements, architecture, data design, and verification.

2.2 Definitions

Term	Definition
EMR	Electronic Medical Record; a digital record of a patient's clinical information and care history.
Patient chart	The longitudinal clinical context for a patient, including demographics, status, relevant history, visits, and clinical information.
Appointment	A scheduled interaction between a patient and a provider/service at a specified time.

Encounter	The clinical interaction in which assessment, examination, diagnosis, treatment, and related documentation are recorded.
Clinical document	A medical report, clinical note, or other record intended to document care.
Finalized document	A clinical document whose content has been completed and protected from ordinary editing.
Amendment	An authorized correction or addition to a finalized clinical document that preserves the original content and records the change.
Walk-in	A patient interaction that begins without a previously scheduled appointment.
RBAC	Role-Based Access Control, in which access is governed by assigned roles and permissions.

3 Product Vision

The product should make the common clinical workflow understandable, connected, and reliable:

Patient → Appointment → Visit/Encounter → Clinical Documentation → Longitudinal Patient Record

The patient chart is the main clinical context. Appointments and encounters should connect to the patient, and clinical documentation should remain linked to the appropriate encounter and patient throughout its lifecycle.

The first release should remain deliberately small while avoiding architectural choices that make later clinical expansion unnecessarily difficult.

4 Scope

4.1 In Scope

- Patient registration, unique identification, search, profile, and clinical summary.
- Relevant patient history, allergies, medications, vitals, alerts, and current status.
- Deceased-patient status and associated lifecycle handling.
- Appointment creation, viewing, cancellation, rescheduling, history preservation, and check-in.
- Flexible walk-in handling.
- Basic appointment-to-encounter linkage.
- Encounter documentation including complaint, vitals, history, examination, diagnosis, treatment, prescription, and clinical notes.
- Medical report generation and clinical-document finalization.
- Controlled amendment of finalized clinical documentation without silently overwriting the original.
- Authentication, role-based authorization, audit logging, validation, and understandable error handling.

4.2 Deferred / Future Scope

The following are outside the MVP unless later discovery explicitly changes the scope:

- Laboratory information and laboratory-result workflows.
- Pharmacy and medication-dispensing workflows.
- Billing, insurance, and payment processing.
- Radiology and imaging workflows.
- Patient portal and patient-facing messaging.
- AI-assisted diagnosis, summarization, or clinical decision support.
- Dedicated mobile applications.
- Production-grade external interoperability integrations.

AI-assisted capabilities and mobile access were suggested during preliminary clinical feedback and are therefore retained as future opportunities rather than MVP requirements.

5 Stakeholders and User Roles

The MVP uses exactly three primary application roles. Permissions should be implemented through role-based authorization so that the system can refine individual permissions without creating unnecessary new user roles.

Role	Primary responsibilities
Administrator	User/account administration, system configuration, access management, and authorized oversight/audit activities.
Clinical Staff	Patient registration and search support, appointment operations, check-in, vitals, and other permitted non-provider clinical workflow tasks.
Doctor/Clinician	Clinical assessment, history and examination documentation, diagnosis, treatment, prescriptions, clinical notes, medical reports, and authorized patient-status updates.

5.1 Role Boundaries

Clinical actions shall not be granted merely because a user can access a patient record. In particular, diagnosis, treatment, clinical notes, medical reports, and clinical-status changes shall require appropriate clinical authorization. Appointment scheduling permissions may be assigned according to configured role permissions rather than being hard-coded to a separate receptionist role.

6 Clinical Workflow

The MVP follows an end-to-end workflow rather than treating patient, appointment, and medical-record features as unrelated CRUD modules.

1. An authorized user registers or searches for the patient.

2. The patient's chart provides the clinical context, including demographics, status, relevant history, allergies, medications, alerts, and previous visits.
3. An authorized user creates or views an appointment when the patient is scheduled.
4. The patient is checked in, or a supported walk-in workflow is initiated when no appointment exists.
5. A visit/encounter is created or accessed and linked to the patient and, when applicable, the appointment.
6. Clinical staff may record permitted preliminary clinical information such as vitals.
7. The Doctor/Clinician records the clinical assessment, examination, diagnosis, treatment, prescription, and clinical notes as applicable.
8. A medical report or other clinical document may be generated and finalized.
9. Once finalized, the document is protected from ordinary editing. An authorized correction is represented through an amendment that preserves the original.
10. The resulting encounter and documents remain part of the patient's longitudinal record.

Walk-in handling shall remain flexible because the appropriate path may depend on patient condition and urgency. The system shall not impose one universal walk-in process that assumes every walk-in should first become a normal scheduled appointment.

7 Requirements Principles

The requirements follow these principles:

- Clinical workflow takes precedence over isolated CRUD operations.
- Patient and encounter linkage is a core integrity requirement.
- Historical clinical information should not be silently destroyed.
- Finalized clinical documents require stronger integrity controls than ordinary editable data.
- Access shall follow least privilege and role-based authorization.
- Requirements shall be traceable to research, discovery, validation, or explicit project decisions.
- Features observed in another EMR are not automatically requirements for Open Clinical Record.
- Future interoperability should be considered in the data and API boundaries without expanding the MVP into a full interoperability implementation.

8 Functional Requirements

The following requirements define the current MVP behavior. The word "shall" indicates a mandatory system behavior.

8.1 Patient Management

ID	Requirement	Description
FR-PAT-001	Register patient	The system shall allow an authorized user to create a patient record using required demographic and administrative fields.
FR-PAT-002	Assign patient identifier	The system shall assign or record a unique patient identifier for each patient record.
FR-PAT-003	Search patient	The system shall allow authorized users to search for patients using supported identifying fields.
FR-PAT-004	View patient profile	The system shall display patient demographics and current status to authorized users.
FR-PAT-005	View clinical summary	The system shall provide an overview of relevant conditions, previous diagnoses, allergies, medications, vitals, and prior visits where data exists.
FR-PAT-006	Record allergies	The system shall allow users with appropriate permission to record and update patient allergies.
FR-PAT-007	Record medications	The system shall allow users with appropriate permission to record relevant medication information.
FR-PAT-008	Display alerts	The system shall surface clinically relevant alerts, including allergies, critical conditions, medications/adverse reactions, follow-up items, and precautions when recorded.
FR-PAT-009	Record patient status	The system shall maintain the patient's current status and restrict status changes according to role permissions.
FR-PAT-010	Mark deceased patient	The system shall allow an authorized clinical user to record deceased status together with the date of death and required supporting information.
FR-PAT-011	Handle deceased patient	Once a patient is recorded as deceased, the system shall prevent inappropriate future appointment activity and flag or cancel affected future appointments according to configured policy.

8.2 Appointment Management

ID	Requirement	Description
FR-APT-001	Create appointment	The system shall allow an authorized user to create an appointment for an eligible patient with required date, time, provider/service, and appointment details.
FR-APT-002	View appointments	The system shall allow authorized users to view scheduled appointments according to their permissions.
FR-APT-003	Reschedule appointment	The system shall allow an authorized user to reschedule an eligible appointment and shall preserve the fact that it was rescheduled.

FR-APT-004	Cancel appointment	The system shall allow an authorized user to cancel an eligible appointment without deleting its historical record.
FR-APT-005	Preserve appointment history	The system shall retain cancelled and rescheduled appointment records with an appropriate status and relevant history.
FR-APT-006	Check in patient	The system shall allow authorized clinical staff to record patient check-in for an eligible appointment or supported walk-in workflow.
FR-APT-007	Support walk-in workflow	The system shall support walk-in registration/check-in without assuming that every walk-in follows the same scheduling path.
FR-APT-008	Link appointment to encounter	The system shall associate a completed check-in/visit with the corresponding appointment when an appointment exists.
FR-APT-009	Prevent deceased-patient booking	The system shall prevent creation of inappropriate future appointments for patients recorded as deceased.

8.3 Encounter and Clinical Documentation

ID	Requirement	Description
FR-ENC-001	Start encounter	The system shall allow an authorized clinical user to start or access an encounter associated with a visit.
FR-ENC-002	Record chief complaint	The system shall allow the Doctor/Clinician to record the patient's presenting complaint or reason for visit.
FR-ENC-003	Record vitals	The system shall allow authorized Clinical Staff to record relevant vital signs for an encounter.
FR-ENC-004	Record history	The system shall allow the Doctor/Clinician to document relevant medical/history information for the encounter.
FR-ENC-005	Record examination	The system shall allow the Doctor/Clinician to document examination findings.
FR-ENC-006	Record diagnosis	The system shall allow an authorized Doctor/Clinician to record the encounter diagnosis or assessment.
FR-ENC-007	Record treatment	The system shall allow an authorized Doctor/Clinician to document treatment or the care plan.
FR-ENC-008	Record prescription	The system shall allow an authorized Doctor/Clinician to record prescription information when applicable.
FR-ENC-009	Create clinical note	The system shall allow an authorized Doctor/Clinician to create and save clinical notes associated with the encounter.
FR-ENC-010	Maintain encounter linkage	The system shall associate clinical documentation with the correct patient and encounter.

8.4 Medical Reports and Document Integrity

ID	Requirement	Description
FR-REC-001	Generate medical report	The system shall allow an authorized Doctor/Clinician to generate a medical report from relevant patient and encounter information.
FR-REC-002	Finalize clinical document	The system shall allow an authorized Doctor/Clinician to finalize a medical report or other clinical document.
FR-REC-003	Restrict finalized editing	The system shall prevent ordinary editing of a finalized clinical document.
FR-REC-004	Amend finalized document	The system shall allow an authorized Doctor/Clinician to create a correction/amendment to a finalized document when permitted.
FR-REC-005	Preserve original document	The system shall retain the original finalized content when an amendment is made and shall not silently overwrite it.
FR-REC-006	Record amendment history	The system shall retain amendment/correction history, including the responsible user and relevant date/time information.
FR-REC-007	View document status	The system shall clearly indicate whether a clinical document is draft, finalized, amended, or another configured lifecycle state.

8.5 Authentication, Authorization, and Auditability

ID	Requirement	Description
FR-SEC-001	Authenticate user	The system shall require authentication before granting access to protected clinical functions and data.
FR-SEC-002	Enforce role permissions	The system shall enforce access according to the authenticated user's assigned role and permissions.
FR-SEC-003	Restrict clinical actions	The system shall restrict diagnosis, treatment, clinical notes, patient-status changes, and medical-report actions to authorized clinical users.
FR-SEC-004	Audit sensitive actions	The system shall record auditable events for significant actions affecting patient records and clinical documentation.
FR-SEC-005	Protect unauthorized access	The system shall deny access to protected records and operations when the user lacks the required permission.

8.6 Validation and Error Handling

ID	Requirement	Description
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FR-VAL-001	Validate required data	The system shall validate required fields and applicable business rules before saving or finalizing records.
FR-VAL-002	Prevent invalid appointments	The system shall reject appointment data that violates configured scheduling or patient-status rules.
FR-VAL-003	Report failures clearly	The system shall present understandable error feedback when an operation fails without exposing unnecessary sensitive information.
FR-VAL-004	Preserve data on failure	The system shall avoid partial or silent loss of clinical data when a save or network operation fails, to the extent supported by the architecture.

9 Business Rules

ID	Rule
BR-001	Cancelled appointments shall remain part of appointment history and shall be visibly marked as cancelled.
BR-002	Rescheduled appointments shall retain sufficient history to distinguish the original scheduling event from the updated appointment.
BR-003	A finalized medical report shall not be silently overwritten. Corrections shall be represented as amendments/corrections with traceable history.
BR-004	Diagnosis, treatment, clinical notes, medical reports, and patient clinical-status changes shall be protected by role-based authorization.
BR-005	Patient alerts shall remain visible where clinically relevant and shall include recorded allergies and other configured high-priority risks.
BR-006	A deceased patient shall not receive inappropriate new future appointments; affected future appointments shall be handled according to configured policy.
BR-007	Walk-in handling shall remain flexible because the appropriate workflow may depend on patient condition and urgency.
BR-008	Clinical information shall remain linked to the correct patient and encounter throughout its lifecycle.

10 Non-Functional Requirements

10.1 Security and Privacy

ID	Requirement
NFR-SEC-001	All protected clinical functions shall require successful user authentication.
NFR-SEC-002	Every protected clinical operation shall be checked against the authenticated user's role/permissions before execution.
NFR-SEC-003	Users shall receive only the permissions required for their assigned responsibilities; clinical permissions shall not be granted by default to non-clinical roles.

NFR-SEC-004	The system shall terminate or invalidate inactive authenticated sessions according to a configured timeout policy.
NFR-SEC-005	User passwords shall never be stored in plaintext and shall be stored using an accepted one-way password-hashing mechanism.
NFR-SEC-006	Patient information shall only be exposed through authorized application functions and shall not be included in unnecessary error messages or logs.

10.2 Data Integrity and Clinical Record Safety

ID	Requirement
NFR-DAT-001	Each clinical record shall remain associated with exactly the intended patient and, where applicable, encounter.
NFR-DAT-002	A failed create/update operation shall not leave a partially committed clinical record.
NFR-DAT-003	Finalized clinical documents shall remain unchanged unless an authorized amendment process is used.
NFR-DAT-004	Audit events for sensitive clinical changes shall identify the acting user and date/time and shall correspond to the affected operation.
NFR-DAT-005	The system shall validate mandatory fields and business constraints before committing affected records.

10.3 Performance

ID	Requirement
NFR-PERF-001	Under normal MVP deployment conditions, common read operations such as patient search, patient profile retrieval, and appointment-list retrieval shall return within 2 seconds for at least 95% of requests.
NFR-PERF-002	Under normal MVP deployment conditions, standard patient, appointment, and encounter save operations shall complete within 3 seconds for at least 95% of requests, excluding external service outages.
NFR-PERF-003	The MVP shall support at least 10 concurrent authenticated users without violating the response targets above.

10.4 Reliability and Network Failure Handling

ID	Requirement
NFR-REL-001	The system shall detect failed save or network operations and shall provide clear feedback rather than reporting a failed operation as successful.
NFR-REL-002	The system shall avoid duplicate creation when a retry is performed after an uncertain network result, where the architecture supports idempotent operation handling.
NFR-REL-003	The system shall preserve successfully committed clinical data when a subsequent network or application operation fails.
NFR-REL-004	Recovery procedures shall be documented for service or database interruption before production deployment.

10.5 Auditability

ID	Requirement
NFR-AUD-001	Sensitive patient-record and clinical-document actions shall generate auditable events.
NFR-AUD-002	Audit records shall contain sufficient information to identify the acting user, affected record or operation, action, and date/time.
NFR-AUD-003	Audit information shall be protected against unauthorized modification or deletion through application permissions.

10.6 Usability

ID	Requirement
NFR-USE-001	The system shall present patient identity and key clinical alerts clearly enough to reduce the risk of selecting or documenting against the wrong patient.
NFR-USE-002	Appointment status shall clearly distinguish scheduled, cancelled, rescheduled, checked-in, and other configured states.
NFR-USE-003	Finalized and amended clinical documents shall have visually distinguishable status indicators.
NFR-USE-004	Validation and error messages shall identify the affected action or field and provide actionable guidance where practical.

10.7 Maintainability and Extensibility

ID	Requirement
NFR-MNT-001	The implementation shall keep clinical domain rules separate from presentation concerns so that future clients or interfaces can reuse core behavior.
NFR-MNT-002	The architecture shall provide clear boundaries between patient, appointment, encounter, clinical-document, authorization, and audit concerns.
NFR-MNT-003	Requirements and architecture decisions shall remain documented so that significant design changes can be traced back to project evidence.
NFR-MNT-004	The system shall be designed so that future clinical modules can be added without changing the fundamental identity and linkage of existing patient and encounter records.

10.8 Interoperability Readiness

ID	Requirement
NFR-INT-001	The internal data model shall use stable identifiers for patients, appointments, encounters, and clinical documents.
NFR-INT-002	Clinical data shall be represented with explicit domain concepts and relationships rather than relying solely on unstructured text fields.

NFR-INT-003	API and domain boundaries shall avoid unnecessary coupling to a single user interface so that future standards-based interoperability can be introduced.
NFR-INT-004	The MVP shall remain FHIR-ready in its conceptual boundaries without requiring full FHIR implementation as part of the initial release.

10.9 Backup and Recovery

ID	Requirement
NFR-REC-001	The deployment shall provide a documented method for backing up persistent clinical data.
NFR-REC-002	Backup procedures shall be tested before production use so that backups are not assumed to be recoverable without verification.
NFR-REC-003	Recovery procedures shall preserve patient, encounter, appointment, clinical-document, and audit relationships.
NFR-REC-004	Backup copies shall be protected against unauthorized access because they contain sensitive patient information.

11 Data Requirements

The detailed database schema is intentionally deferred to the data-design stage. The following logical entities and relationships are requirements-level constraints for that design.

Entity	Requirements-level responsibility
Patient	Unique patient identity, demographics, status, and longitudinal linkage to clinical information.
Patient Alert	Clinically relevant warnings such as allergies, critical conditions, adverse reactions, follow-up items, and precautions.
Medication Information	Relevant current or historical medication information associated with the patient and/or encounter as appropriate.
Appointment	Scheduled patient interaction, provider/service, time, status, and historical scheduling information.
Encounter	Clinical visit context linked to a patient and optionally to an appointment.
Clinical Note	Clinician-authored documentation associated with an encounter.
Medical Report / Clinical Document	Report or other clinical document with lifecycle status and finalization/amendment behavior.
Document Amendment	Authorized correction associated with an original finalized document while preserving the original.
User	Authenticated application account with an assigned role and permissions.
Audit Event	Record of significant security, patient-record, and clinical-document actions.

11.1 Core Relationship Constraints

- Each patient shall have a unique identifier.
- Each encounter shall identify the patient to whom it belongs.

- An appointment may be linked to an encounter when the scheduled interaction results in a visit.
- A walk-in encounter may exist without a prior appointment.
- Clinical documents shall identify the patient and, where applicable, the encounter from which they originated.
- An amendment shall identify the finalized document it corrects and shall not replace the original content.
- Sensitive actions shall be attributable to an authenticated user through audit information.

12 Constraints and Assumptions

12.1 Constraints

- The internship MVP has a deliberately limited scope and shall not attempt to implement every EMR module.
- Development and deployment choices should favor maintainable and accessible technologies suitable for the internship environment.
- The system must treat patient information as sensitive data and apply appropriate access controls.
- External service or network outages cannot be assumed to be absent; failure handling is therefore part of the requirements.

12.2 Assumptions

- A deployment environment will provide persistent storage for application data.
- Authorized users will have accounts associated with one of the three MVP roles.
- Exact clinical terminology, mandatory fields, and operational policies may require additional validation with clinical stakeholders.
- The MVP will not independently provide clinical decision-making; it records and presents information entered by authorized users.

13 Clinical Validation Evidence

A preliminary survey response was received from a Doctor/Clinician. The response supports the inclusion of patient demographics, conditions, previous diagnoses, vitals, allergies, medications, previous visits, configurable appointment permissions, preservation of cancelled and rescheduled appointments, core encounter information, clinician-controlled diagnosis/treatment/notes/reports/status changes, amendment rather than silent replacement of finalized reports, clinical alerts, and deceased-patient handling.

The respondent also identified network reliability as a practical concern. AI-assisted capabilities and mobile access were suggested as future opportunities rather than immediate MVP requirements.

This evidence is preliminary and is not treated as statistical validation. Additional clinician or stakeholder review should be used to confirm workflow details and resolve remaining policy decisions before a final production baseline.

14 Acceptance and Verification Strategy

Each functional requirement should be verified through an appropriate combination of inspection, unit testing, integration testing, workflow testing, and stakeholder review.

Area	Verification approach
Patient management	Test patient creation, unique identification, search, profile, summary, alerts, status, and deceased lifecycle behavior.
Appointments	Test creation, viewing, cancellation, rescheduling, history preservation, check-in, walk-in handling, encounter linkage, and deceased-patient restrictions.
Encounter	Test creation and linkage plus recording of complaint, vitals, history, examination, diagnosis, treatment, prescription, and notes.
Clinical documents	Test generation, finalization, edit restriction, amendment, original preservation, amendment history, and status display.
Security	Test authentication, role permissions, denial of unauthorized actions, session behavior, password handling, and sensitive-action audit events.
Data integrity	Test patient/encounter linkage, transaction behavior, failed saves, finalized-document preservation, and validation constraints.
Performance	Measure representative patient, appointment, and encounter operations under the defined MVP load assumptions.
Reliability	Simulate failed network/save operations and verify clear feedback, no false success, and no unintended data loss or duplication.
Usability	Conduct scenario-based review of patient identification, alerts, appointment status, document status, and validation feedback.

15 Traceability

The requirements are supported by the dedicated traceability matrix at:

<https://github.com/elan-thinks/open-clinical-record/blob/main/docs/03-requirements/traceability-matrix.md>

The matrix connects clinical discovery and workflow use cases to functional requirements, business rules, non-functional requirements, and verification expectations. It also records role coverage and evidence sources.

The principal use cases are:

- UC-01 Patient registration and identification
- UC-02 Patient search, chart, and clinical summary
- UC-03 Appointment scheduling and management
- UC-04 Check-in and walk-in handling
- UC-05 Clinical encounter and assessment
- UC-06 Clinical documentation and reporting

- UC-07 Finalization, correction, and amendment of clinical documents
- UC-08 Patient status and deceased-patient lifecycle
- UC-09 Authentication, authorization, and audit

16 Open Decisions for Review

The following items should be confirmed during mentor and stakeholder review rather than hidden as assumptions:

- Exact required demographic and identification fields for patient registration.
- Exact appointment status values and rescheduling-history representation.
- Exact policy for future appointments when a patient is marked deceased, including whether they are automatically cancelled or flagged for authorized review.
- Exact clinical-document amendment permissions and required amendment reason fields.
- Exact session-timeout value and deployment-specific performance environment.
- Backup frequency, retention, and recovery-time expectations for the eventual deployment.
- Final terminology and clinical-field validation with additional clinical stakeholders.

These decisions do not prevent architecture work; they should be represented as explicit configurable or reviewable decisions where appropriate.

17 Future Expansion Considerations

The MVP should leave clean boundaries for future modules without implementing them prematurely. Likely expansion areas include laboratory, pharmacy, billing, radiology, patient-facing access, mobile access, AI-assisted functionality, and standards-based interoperability.

Future functionality should build on the existing patient, appointment, encounter, clinical-document, authorization, and audit concepts rather than introducing parallel representations of the same clinical facts.

18 References and Project Evidence

The SRS is based primarily on the project's internal research and discovery documents:

- Clinical workflow discovery: `docs/02-discovery/clinical-workflow.md`
- Patient-chart research: `docs/01-research/patient-chart-research.md`
- Appointment-scheduling research: `docs/01-research/appointment-scheduling-research.md`
- Clinical-documentation research: `docs/01-research/clinical-documentation-research.md`
- Research log: `docs/01-research/research-log.md`
- Requirements traceability matrix: `docs/03-requirements/traceability-matrix.md`
- Architecture decision record: `docs/04-architecture/adr/0001-record-architectural-decisions.md`

External standards and implementation references should be incorporated into the architecture and interoperability documentation as those decisions are made. The SRS intentionally avoids treating external EMR feature inventories as requirements unless supported by project evidence.

19 Requirement Baseline Statement

This document represents the first complete requirements draft for the Open Clinical Record MVP. It establishes a coherent and traceable baseline for moving into architecture and data-model design while clearly identifying remaining decisions and the limits of the preliminary clinical validation.